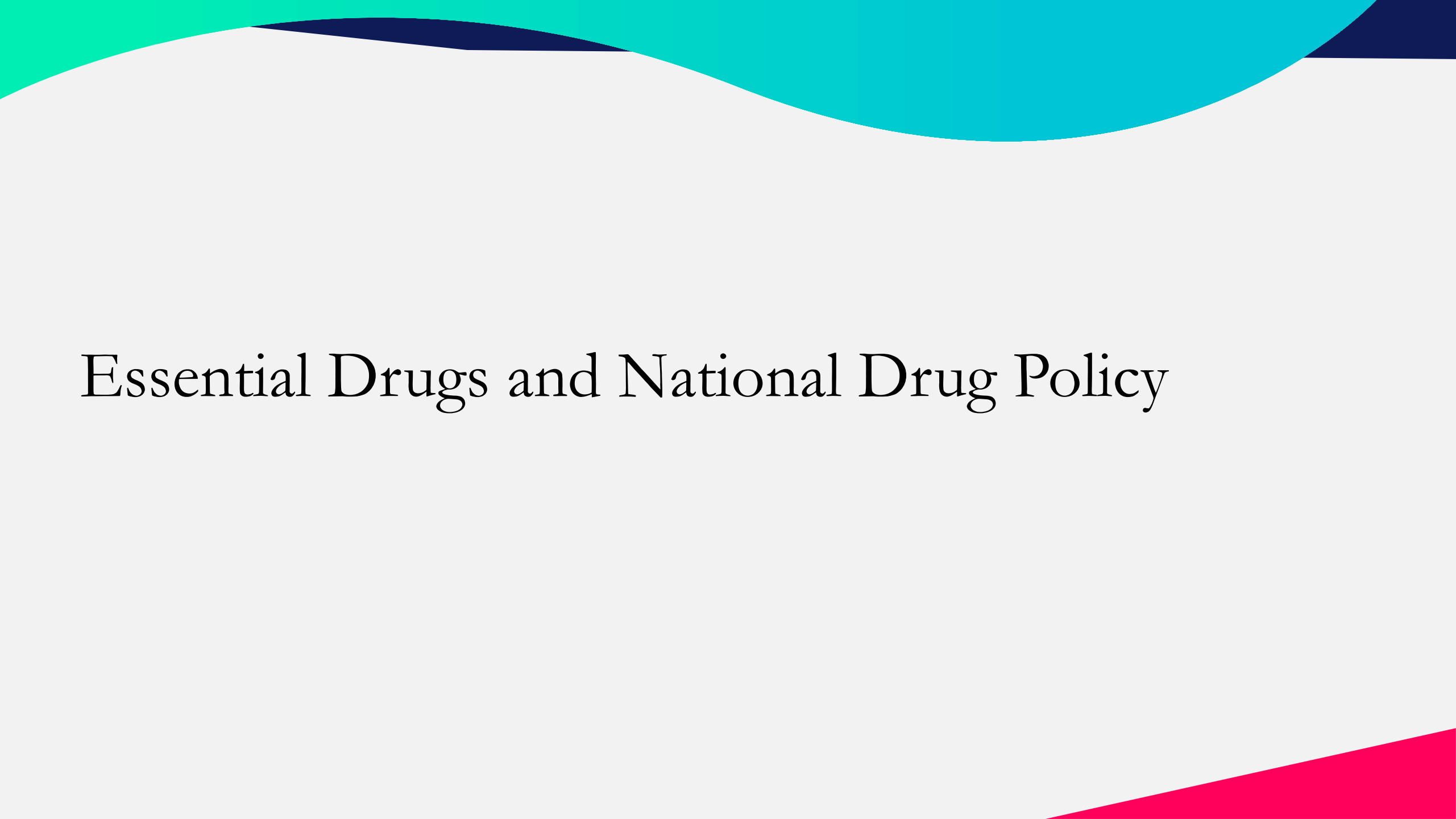




Essential Drugs and National Drug Policy

Content

- Introduction to National Drug Policy
- Development and implementation of National Drug Policy
- Objectives and key strategies of NDP
- Concept of essential medicine
- Formulary process
- Pharmaceutical Sector Transformation Plan of Ethiopia

National drug policy

- ♠ A national drug policy is a commitment to a goal and a guide for action.
- ✓ A national drug policy, presented and printed as an official government statement
- ✓ The policy document should be developed through a systematic process of consultation with all interested parties.

Cont . . .

Why is a national drug policy needed?

- ✎ A national drug policy is needed for many reasons. The most important are:
 - ✎ to present a formal record of values, aspirations, aims, decisions and medium- to long-term government commitments;
 - ✎ to define the national goals and objectives for the pharmaceutical sector, and set priorities;
 - ✎ to identify the strategies needed to meet those objectives, and identify the various actors responsible for implementing the main components of the policy;
 - ✎ to create a forum for national discussions on these issues.

Objectives of a national drug policy

01

Access

Equitable availability and affordability of essential drugs

02

Quality

The quality, safety and efficacy of all medicines

03

Rational use:

The promotion of therapeutically sound and cost-effective use of drugs by health professionals and consumers

Cont . . .

Access

♠ Physical availability

- The **type and quantity of product or service needed and provided**

♠ Affordability

- Users ability to pay for the products/services

♠ Geographical accessibility

- The location of the product/service and eventual user

Cont . . .

Quality

- ♠ *Quality is simply meeting the customer's requirements*
- ♠ The totality of features and characteristics of a product or service that bear on its ability to satisfy stated or implied needs

Cont . . .

Components of a national drug policy, linked to key policy objectives

Objectives:	Access	Quality	Rational use
Components:			
Selection of essential drugs	X	(X)	X
Affordability	X		
Drug financing	X		
Supply systems	X		(X)
Regulation and quality assurance		X	X
Rational use			X
Research	X	X	X
Human resources	X	X	X
Monitoring and evaluation	X	X	X

X = direct link; (X) = indirect link



Cont . . .

1. **Selection of essential drugs**

- No public sector or health insurance system can afford to supply or reimburse all drugs that are available on the market

2. **Affordable:** prices are an important prerequisite for ensuring access to essential drugs in the public and private sectors.

3. **Drug financing:** increased government funding for priority diseases, and the poor and disadvantaged;



Cont . . .

4. Supply System

- Public-private mix in drug supply and distribution systems;
- Commitment to good pharmaceutical procurement practices in the public sector; Inventory control, and prevention of theft and waste; disposal of unwanted or expired drugs.

5. The **drug regulatory authority** is the agency that develops and implements most of the legislation and regulations on pharmaceuticals, to ensure the quality, safety and efficacy of drugs, and the accuracy of product information.



Cont . . .

5. The **rational use** of drugs means that patients receive medicines appropriate for their clinical needs, in doses that meet their individual requirements, for an adequate period of time, and at the lowest cost to them and their community.





Cont . . .

6. Research: facilitates the implementation, monitoring and evaluation of different aspects of drug policy.

- ♠ Assessing the drug policy's impact on national health service systems and delivery,
 - ♠ Studying the economics of drug supply,
 - ♠ Identifying problems related to prescribing and dispensing, and
 - ♠ In understanding the sociocultural aspects of drug use.
- 



Cont . . .

6. Human resources development includes the policies and strategies chosen to ensure that there are enough trained and motivated personnel available to implement the components of the national drug policy.

7. Monitoring and evaluation: monitoring of the pharmaceutical sector through regular indicator-based surveys;



How to develop the NDP?

Overview of the national drug policy process

- ✍ a complex process of development, implementation & monitoring.
- *Throughout the process*
 - ☞ careful **planning**
 - ☞ **involvement of all parties**
 - ☞ the **political dynamics**

Cont. . .

Planning

A drug policy without an implementation plan remains a dead document.

Types of plan

1. **Strategic plan** to develop the policy itself, which should specify the various steps in the development process, like which stakeholders and how many to involve.
2. **Implementation plan**, or master plan, details the various activities for each component of the policy.
 - Covers a 3-5-year period.

Cont. . .

- The implementation plan spells out what needs to be done and who is responsible, estimates the budget and proposes a time frame. If resources are insufficient without external input, a set of priority activities should be identified that can be executed within the existing means.

Involvement of all parties

There should be consultation, dialogue and negotiations with all interested groups and stakeholders.

Stakeholders

- Ministries of (education, trade, industry)
- Doctors, pharmacists and nurses,
- Pharmaceutical industries (local and international)
- Drug sellers, academia, NGOs,
- Professional associations
- Consumer groups,
- Traditional and herbal medicine practitioners,
- The drug regulatory agency
- Insurance companies

Political dynamics

- A policy usually seeks to achieve equity of access to basic health care, primarily by making the pharmaceutical sector more efficient, cost-effective and responsive to health needs.
- Given the diverse interests and the economic importance of the issues involved, opposition to the new policy and attempts to change it during implementation can be expected.
- Strong political leadership and sustained commitment are vital

Formulating a NDP

Step 1: Organize the policy process

- The ministry of health take the lead role in formulating a NDP.
- At this stage identify:
 - The structure of the policy
 - Major objectives
 - Priority components.
 - All the interested parties
 - The necessary resources and how these can be obtained.

Cont . . .

- **Step 2: Identify the main problems**

- ✓ Bring together a small team of experts,
- ✓ Do situation analysis
 - ✓ To identify the main problems, to make recommendations about what needs to be done and what can be done, and to identify possible approaches.

- **Step 3: Make a detailed situation analysis**

- ✓ to further analyze the **source of the problems**,
- ✓ identify potential **solutions**
- ✓ choose the most appropriate **strategies**
- ✓ set **priorities**
- ✓ serve as a **baseline** for future systems of **M** and **E**.

Cont . . .

Step 4: Set goals and objectives for a national drug policy

Identify the main problems and then set goals and prioritise objectives

Step 5: Draft the text of the policy

Step 6: Circulate and revise the draft policy

- ✓ MoH, other gov't ministries and departments
- ✓ relevant institutions and organizations outside the government
- ✓ revise the draft document based on the received comments and finalized

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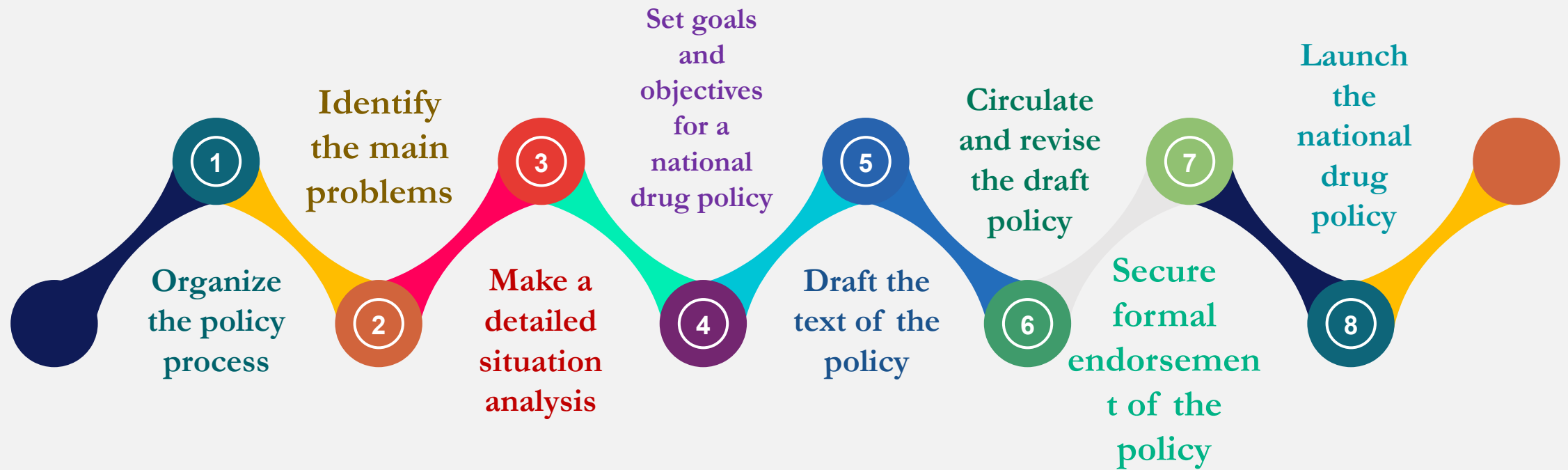
Step 7: Secure formal endorsement of the policy

- ✓ demonstration of political commitment but it can also cause problems, as future adjustments to the policy may become difficult.

Step 8: Launch the national drug policy

- ✓ promote the policy through a clear and well-designed information campaign.
- ✓ Public endorsement by respected experts and opinion leaders.

Summary on steps in formulating NDP



Essential medicine concept

During and soon after **WWII**, a number of pharmaceuticals were introduced to the pharmaceutical market and the whole field of therapeutics was revolutionized.



Many are minor variations of prototype drug and offer no therapeutic advantage over other medicines that are already available.

‘Miracle’ was the word used by many authors to describe the effects.

However, 70 % of pharmaceuticals on the world market are duplicative or nonessential.

Essential medicine concept and its principles

Concept:

- A limited range of carefully selected medicines leads to better health care, better drug management and lower costs.

Pharmaceutical Budget of Ethiopia

- ETB 79.4 billion in 2022/23
- ETB 100.2 billion in 2023/24(12.5%)

Total Budget of Ethiopia

- 2023/2024 fiscal year's budget of 801.6 billion birrs.

Cont. . .

- Health is a fundamental human right.
- In many countries drug costs account for a large share of the total health budget.
- Despite the obvious medical and economic importance of drugs there are still widespread problems with
 - lack of access,
 - poor quality,
 - irrational use and waste.

Essential medicine principles

01

The **vast majority of health problems** can be treated with a small, carefully selected number of drugs.

02

Most practitioners routinely use fewer than 200 drugs

03

Supply activities can be carried out most efficiently for a limited number of pharmaceutical products.

04

Patients can be better informed when confronted with limited drugs.

- ✓ Doctors & travelers carried limited drugs and satisfied basic need of the community where they go.
- ✓ The **basic list idea**, whereby a limited range of affordable drugs used to meet basic needs of whole populations, had been in place for the first time before the WWII, in Norway and was successful. Later on Papua New Guinea (early 1950's), Sri Lanka (1959), and Cuba (1963) followed.

Cont . . .

- WHO adopted the concept with the view that essential medicines should be given priority.
- In 1975- WHO defined essential medicines
- In 1977, WHO published the first model list of essential drugs
 - List is revised every 2-3 years by WHO expert committee
- In 1978, historic goal of 'health for all by 2000'essential medicines were recognized as one of the eight elements of primary health care.

Alma-Ata Declaration (1978)

Health For All by the year 2000

1. Health education on prevailing health problems and the methods of preventing and controlling
2. Nutritional promotion including food supply
3. Supply of adequate safe water and sanitation
4. Maternal and child health care
5. Immunization against major infectious diseases
6. Prevention and control of locally endemic diseases
7. Appropriate treatment of common diseases and injuries
8. Provision of essential drugs.

Cont . . .

Essential drugs concept

- *“a limited number of carefully selected drugs based on agreed clinical guidelines leads to more rational prescribing, to a better supply of drugs and to lower costs”.*

Selection criteria: Essential medicines are selected with due regard to disease prevalence, evidence on efficacy and safety, and comparative cost-effectiveness

Cont . . .

- **Purpose:** Essential medicines are intended to be **available** within the context of functioning health systems **at all times**, in **adequate amounts**, in the **appropriate dosage forms**, with **assured quality**, and **at a price** the individual and the community can afford.
- **Implementation:** The implementation of the concept of essential medicines is intended to be flexible and adaptable to many different situations; exactly which medicines are regarded as essential remains a national responsibility.

Summary of advantages of a limited list of essential medicines



Generic names

- The drugs on the essential drugs list are referred to by their INN or generic names.
- The generic name is used in writing prescriptions as well as in purchasing drugs.
- The use of the generic name for these purposes has certain advantages:
 - ✓ There is easy recognition of the type of drug, especially where many selected drugs exist in that class (e.g. all benzodiazepines have INNs ending with “-zepam”);
 - ✓ Drugs can be purchased from multiple sources, thus giving the advantage of buying at a competitive price;
 - ✓ **Product substitution** is easy where similar **bio-availability is confirmed**;
 - ✓ The confusion associated with the use of brand names can be avoided.

The formulary process

- A *formulary* is a continually updated list of medications and related information, representing the clinical judgment of physicians, pharmacists, and other experts in the diagnosis, prophylaxis, or treatment of disease and promotion of health.

Cont . . .

- A *formulary system* is the ongoing process through which a health care organization establishes policies regarding the use of drugs, therapies, and drug-related products and identifies those that are most medically appropriate and cost-effective to best serve the health interests of a given patient population.

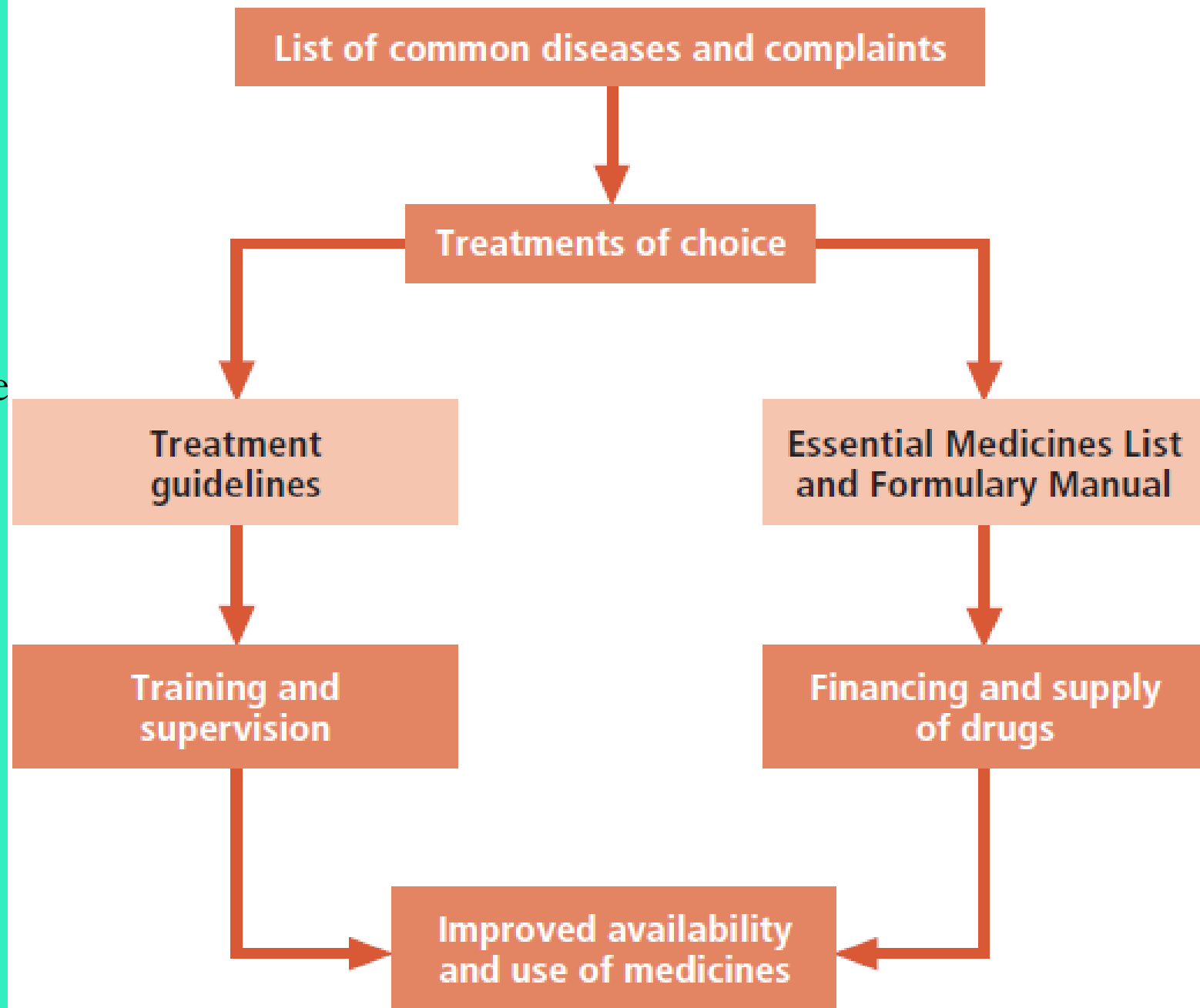
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The formulary process consists of **preparing, using and updating a**

- ***Formulary list or essential medicines list (EML)***
- **Formulary manual**
- **Standard treatment guidelines (STGs).**

How STGs and EMLs lead to better prevention and care

Health systems should develop, maintain, and implement a formulary management process. Decisions on the management of a formulary system should be founded on the evidence-based clinical, ethical, legal, social, philosophical, quality-of-life, safety, and economic factors that result in optimal patient care



Cont . . .

- **Advantages of EML**

1. Cost containment: The procurement: Economies of scale.
2. Enhanced equity in access to essential medicines:
3. Improved quality of care
4. Quality assurance, procurement, storage, distribution and dispensing are all easier with a reduced number of drugs.
5. Training of health workers and drug information in general can be more focused, and
6. Prescribers gain more experience with fewer drugs and are more likely to recognize drug interactions and adverse reactions

PRINCIPLES OF FORMULARY LIST MANAGEMENT

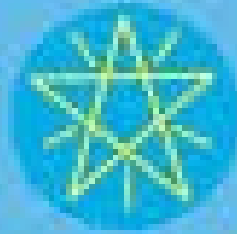
- ♠ Select drugs according to the needs of patients
- ♠ Select drugs of choice for the conditions identified
- ♠ Avoid duplications, both therapeutic and pharmaceutical (dosage forms)
- ♠ Use explicit selection criteria, based on proven efficacy, safety, quality and cost
- ♠ Use evidence-based information whenever possible
- ♠ Be consistent with national EMLs and STGs

Cont . . .

- ♠ Consider requests for the **addition** of new drugs only when made by health-care staff, not by the pharmaceutical industry
- ♠ Require that requests for the addition of new drugs are justified using **documented evidence on efficacy**,
 - ♠ Relative efficacy, safety and comparative cost-effectiveness and that the person requesting any new drug **declare any conflict of interest**
- ♠ Carry **out annual systematic reviews** of all therapeutic classes to avoid duplication.

Formulary manual

- ♠ The formulary manual is the publication that brings all the important summary information on medicines in the formulary list together in a manual.
- ♠ It contain an *alphabetically* and *therapeutically* arranged listing of all the formulary drugs.
- ♠ Is drug centered



ETHIOPIAN MEDICINES FORMULARY

Second Edition

Food, Medicine and Healthcare
Administration and Control Authority of Ethiopia

2013

Basic information about each medicine

- Generic name
- Dosage and strengths
- Indications, CI and precautions
- Side-effects
- Dosage schedule
- Instructions and warnings
- Drug, food, laboratory interactions

Supplementary information for medicines

- Price
- Regulatory category
- Storage guidelines
- Patient counseling information
- Labeling information
- Brand names and synonyms

Standard treatment guidelines

- ♠ STGs or treatment protocols are a proven, effective strategy to **promote appropriate prescribing**, when used in conjunction with educational strategies to promote their use.
- ♠ STGs may be defined as *‘systematically developed statements to help practitioners or prescribers make decisions about appropriate treatments for specific clinical conditions’* (MSH 1997).



Food, Medicine and Healthcare
Administration and Control
Authority of Ethiopia

Standard Treatment Guidelines
For General Hospital



As a minimum, they should contain information on

- ✓ Clinical features,
- ✓ Diagnostic criteria,
- ✓ Non-drug and drug treatments (first-, second-, third-line), and
- ✓ Referral criteria.
- ✗ It is **disease centered**

Cont . . .

STGs are very useful in:

- ✂ **Providing guidance** to health professionals on the diagnosis and treatment of specific clinical conditions
- ✂ **Orienting new staff** about accepted norms in treatment
- ✂ Providing prescribers with **justification for prescribing decisions** made in accordance with STGs
- ✂ Providing a **reference point** by which to judge the quality of prescribing
- ✂ **Aiding efficient estimation of drug needs** and setting priorities for procuring and stocking drugs. (Morbidity Method)

Cont . . .

The problems associated with STGs include:

- ✍ A development process which is **difficult**, **time-consuming**, and requires human and financial resources
- ✍ The need to **update regularly** to avoid STGs becoming obsolete
- ✍ The danger of **inaccurate or incomplete guidelines** which provide wrong information to prescribers, so **doing more harm** than good.